AC-16 Wearable Cardioverter-Defibrillator Vest

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for a patient wearing a ZOLL LifeVest or another wearable cardioverter-defibrillator who is symptomatic, shocked, has a device alarm, or is in cardiac arrest.

LifeVest warning, handoff, and EMS takeover

1. Assess patient + alert	Standard assessment; determine whether device is silent, has a single gong, announces treatment was delivered, or produces a two-tone treatment siren.
2. Active treatment siren?	If prompt says response buttons should be pressed or bystanders must not interfere, a shock is likely.
3. Clear patient	Stop contact/CPR briefly, clear everyone, and allow device treatment. Only the patient may press response buttons.
4. Siren stops/CPR prompt	Immediately reassess pulse and breathing; start/resume CPR when indicated.
5. EMS takes over	Pull battery first; remove/disconnect vest; clean/dry chest; apply EMS pads; use AC-03/AC-09 when indicated.
6. Pulse present/post-shock	Continuous monitor, 12-lead, assess ACS/dysrhythmia, inspect skin, document shock/symptoms, and transport.

Key treatment and handoff

Item	Approved action
Electrical therapy	Before external defibrillation, cardioversion, or pacing, disconnect the LifeVest monitor from the electrode belt. Keep EMS pads clear of vest electrodes and gel.
Equipment	Bring garment, monitor, battery, charger/hotspot, extra battery, and stored-event information to hospital.

Never touch the patient during the active treatment warning or while a LifeVest shock is being delivered. Resume CPR immediately when the warning ends or the device prompts CPR.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Never press response buttons for the patient; only the patient may use them as a consciousness test.
- If vest is removed, rinse away blue conductive gel with water and dry chest before EMS pad placement.
- Do not delay an indicated EMS shock while troubleshooting, collecting equipment, or contacting technical support.

Linked pathways

Pathway	Use when
AC-03	Adult cardiac arrest
AC-04	ACS/STEMI after shock
AC-07/AC-08	Perfusing ventricular dysrhythmia
AC-09	VF/pulseless VT once EMS assumes defibrillation

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.
- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.
- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.